

Capacity & Scheduling Vendor Demos: Comparison

Axle Health (Sep 22), CareStitch (Sep 23), VitalisCare (Sep 24)

Compassus Home Health
Internal, for discussion

Summary

- **Axle** was the strongest product. It is built for home health and showed recommendations for clinician, day and time. Go-live depends on getting HCHB data to them faster than our current 3-hour refresh.
- **CareStitch** has the best tools for clinicians and for coordinating coverage, but it does not recommend or automate anything yet, and its HCHB integration is not in production.
- **VitalisCare** is a hospice product. Its HCHB integration is live, but authorization, episode/LUPA logic, therapy and patient communication would all need to be built.

Variable coverage (79 variables from the Variable Inventory)

	Capacity (22)	Scheduling (45)	Engagement (12)	Total (79)
Axle	86% / 73%	76% / 51%	83% / 25%	80% / 53%
CareStitch	95% / 73%	62% / 36%	83% / 33%	75% / 46%
VitalisCare	91% / 55%	62% / 38%	92% / 0%	75% / 37%

First number: discussed on the call (includes roadmap items). Second number: shown in the software. Of the 49 MVP variables, Axle showed 59%, CareStitch 49%, VitalisCare 41%.

Not covered by any vendor (9): add-on orders, acuity to skill level, cognitive/dementia constraints, diagnosis-driven timing, missed-visit documentation (all MVP), plus start-time flexibility, lunch/documentation pattern, clinician safety and infection-control sequencing.

Open items for the on-site

Axle	Joint data plan (intake feed for new patients, HCHB refresh target for orders, date to turn on write-back). Run our scenario end to end including LUPA and a referral decision. Partnership terms, company size, 5k+ ADC reference.
CareStitch	Dated HCHB production plan for read and write. Demo of the automation work they mentioned and a date for recommendations. Date for patient text confirmation. Run a 60-day episode.
VitalisCare	Written scope, dates and cost for the home health build. Call-Off Center shown live, including when no one takes the visit. Security and uptime (not covered on the call), company size and funding.

Assessment (1 = low, 5 = high)

	Axle	CareStitch	VitalisCare
Product sophistication	4 - Recommends clinician, day and time and shows the factors behind each. Suggests coverage for call-outs. Capacity forecast by zip.	2 - Filters out ineligible clinicians; the scheduler makes the choice. Recommendations and AI are still in research.	3 - Ranks providers and suggests better visit days with a short explanation. Call-off tool not released.
Reduction in scheduler tasks	4 - Work queues, bulk scheduling of the episode, call-out reassignment in seconds. They claim 74% less scheduler time per visit. Depends on faster HCHB data.	3 - Replaces the Teams/email scramble to find coverage and removes territory lookups. Each assignment is still manual.	2 - Matching helps, but visit plotting stays in HCHB, call-off recovery is not built, and the 3-hour lag risks double entry.
Clinician experience	4 - Clinician optimizes and finalizes the next day; patients get a confirmation text instead of a call. Reasoning for assignments goes to managers, not clinicians.	4 - Designed by clinicians: drive time on the calendar, click-to-call with a call log, care-team chat, offline app, 93% adoption. Clinicians still call every patient.	2 - Routes the day and reroutes as it changes, but tracks location all workday and reviews route adherence. Separate app from PointCare; no patient messaging.

Scores are our read of the demos, not the formal scorecard.

Key differences

	Axle	CareStitch	VitalisCare
HCHB integration	Built but turned off. Needs data within 10-15 min. Intake feed would cover new patients.	Read-only beta, not in production. No write-back.	Live. Reads the log ship, writes by RPA. 3-hour refresh too slow for call-offs.
Home health logic	Visit windows, Medicare week, front-loading, SOC/eval/recert checks. LUPA not shown.	None. Relies on HCHB for rules.	Hospice rules only. Auth, episode, LUPA and therapy to be built.
Patient communication	Two-way text is live. Sends confirmations rather than asking for times.	Click-to-call and call log. Text confirmation on roadmap.	None today.
Customers	~25 orgs, 80% home health, largest 11k ADC	79 orgs, 69% home health, largest 5.3k (one branch)	3 live on Sync, ~6-7k patients, largest 3.5k ADC
Scorecard / next step	63, Conditional. Recommend moving to on-site with a data plan.	56, Conditional. Hold; follow-up call on HCHB and automation.	67, Consider. Hold until home health build is scoped and dated.

Coverage method: each Variable Inventory row was marked shown, discussed or not covered for each call. Shared variables are counted under Capacity; Coordination variables under Engagement. Per-variable detail available on request.